INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Methotrexate: head and neck cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00893
Version 1

intravenous (IV) on Days 1, 8 and 15 every 28 days

Treatment stages

01 **Chemotherapy course**
IV on Days 1, 8 and 15 every 28 days

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Recurrent or advanced head and neck cancer

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

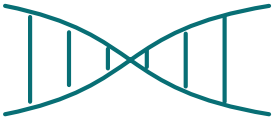
How the medicine works

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

CYTOTOXIC CHEMOTHERAPY

Methotrexate

Intravenous chemotherapy



Blocks folate-dependent deoxyribonucleic acid (the cell's genetic material) (DNA) production

Methotrexate interferes with folate-dependent reactions that cells need to make DNA. This can slow cancer-cell division, while also affecting blood-cell production and the lining of the mouth and bowel.

Folate pathway inhibited → DNA building blocks reduced → Cell division slowed

What to watch for

Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Research supports a treatment discussion; it cannot predict one person's outcome.

The source cites methotrexate experience in recurrent head and neck cancer. Evidence must be interpreted for this exact 40 mg/m² intravenous Days 1, 8 and 15 schedule. Results from other weekly doses or combinations should not be presented as established benefit for this regimen.

CHAPTER 02

Your schedule, tests and appointments

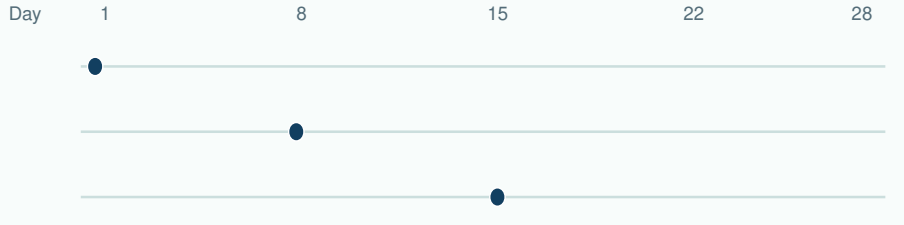
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Methotrexate: head and neck cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

IV ON DAYS 1, 8 AND 15 EVERY 28 DAYS

Chemotherapy course

Repeat as prescribed

Methotrexate 40 mg/m² by intravenous bolus on Days 1, 8 and 15 of each 28-day cycle, until progression or unacceptable toxicity. There is no scheduled Day-22 dose. The clinician may increase to 50 mg/m² only if there has been no mucositis.

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00893 v1: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant's plan; no routine universal tumour-marker panel is specified.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Methotrexate: head and neck cancer: treatment and checks

Recurrent, advanced or metastatic head and neck squamous cell cancer in people with comorbidities or unsuitable for combination chemotherapy.

Methotrexate 40 mg/m² by intravenous bolus on Days 1, 8 and 15 of each 28-day cycle, until progression or unacceptable toxicity. There is no scheduled Day-22 dose. The clinician may increase to 50 mg/m² only if there has been no mucositis.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.	Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.	Keep a current dose and symptom record.	Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant's plan; no routine universal tumour-marker panel is specified.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

IV on Days 1, 8 and 15 every 28 days

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / **act:** Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.

STEP 2

Taking or receiving treatment

Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Your exact treatment schedule

Tests and preparation

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 28-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Methotrexate IV	 **Day 1** Prescribed intravenous dose after safety review.
Methotrexate IV	 **Day 8** Prescribed intravenous dose after safety review.
Methotrexate IV	 **Day 15** Prescribed intravenous dose after safety review.
No scheduled methotrexate	 **Days 16–28** No Day-22 dose. Continue symptom checks and prepare for the next cycle.

Treatment dates depend on safety checks and the individual prescription. Side effects can occur on any day.

The next cycle starts 28 days after this one, if your team confirms it.

> Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Day 1: IV treatment

Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself. Confirm blood results and recovery before a dose.

Day 2–7: Days 2–7: between doses

No scheduled methotrexate on these days. Continue mouth care, symptom checks and prescribed support. Call for new mouth ulcers, fever, bleeding or breathing symptoms.

Day 8: Day 8: IV treatment

Day 9–14: Days 9–14: between doses

Day 15: Day 15: IV treatment

Day 16–28: Days 16–28: between doses

Support around the medicines

1 **Preventing and treating sickness**

Methotrexate in this regimen is low sickness risk. Follow the prescribed prevention/rescue plan.

2 **Medicine-specific support**

3 **Mouth care, nutrition and daily life**

Follow the unit’s mouth-care plan. Report pain, poor intake or weight loss early; ask for dietetic support. Pace activity, rest and work around symptoms.

4 **Medicine and interaction review**

Bring all prescription, non-prescription and herbal medicines. Check with the oncology pharmacist before starting or stopping anything.

Your diagnosis and treatment setting

Specialist tests

Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant’s plan; no routine universal tumour-marker panel is specified.

Treatment aims and choices

Review and alternatives

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval. Discuss suitable alternatives, supportive care and the option of declining treatment.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

> **Try explaining the plan back:** Explain when you take or receive treatment.,Name the tests and next review date.,Show how to contact the team urgently.,Explain what to do if a dose is missed or treatment is held.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 1, 8 and 15: methotrexate

Active medicines: Methotrexate

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg PO OD	Each prescribed methotrexate treatment day (Days 1, 8 and 15) Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	PRN Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.
- Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.
- Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.
- Check interactions before any local adjunct: methotrexate clearance and toxicity can be affected by other medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

Methotrexate

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

Methotrexate

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Blood counts and infection

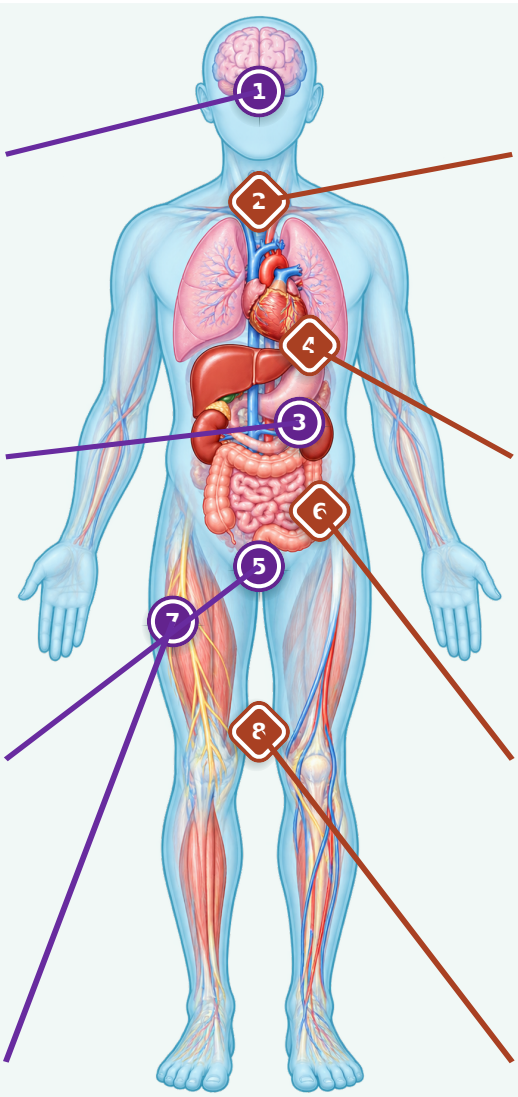
Methotrexate

Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent oncology advice.

4 Breathing

Methotrexate

New cough or breathlessness needs prompt review; serious causes include infection or lung injury.



5 Sickness and bowels

Methotrexate

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

Methotrexate

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Skin, hands and feet

Methotrexate

Report painful palms or soles, rash or blistering early. Severe spreading rash or mouth/eye involvement needs urgent review.

8 Fertility and pregnancy

Methotrexate

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00893 v1: official regimen**
   https://healthservice.hse.ie/documents/6862/893_V1_Methotrexate_40_HNSCC.pdf
2. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
3. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
4. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
5. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf